Name	: Mr. ADITYA PRATAP SINGH	Sample ID	: A4977282
Age/Gender	: 31 Years/Male	Reg. No	: 0032502240042
Referred by	:	Collected On	: 24-Feb-2025 10:30 AM
Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Serum	Reported On	: 24-Feb-2025 07:32 PM
Client Code	: BGLDLP010	Report Status	: Final Report

SEROLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
VDRL- Syphilis Antibodies	Non Reactive		Non Reactive	Slide Flocculation

The serological diagnosis of syphilis is classified into two groups: Nontreponemal tests (RPR/VDRL) and Treponemal tests (TPHA/CLIA). Syphilis serology is a treponemal assay for the qualitative determination of antibodies to T. pallidum in human serum or plasma as an aid in the diagnosis of syphilis. Treponemal tests may remain reactive for life, even following adequate therapy thus a positive result suggests infection with Treponema pallidum but does not distinguish between treated and untreated infections. Therefore, the results of a nontreponemal assay, such as rapid plasma reagin, are needed to provide information on a patient's disease state and history of therapy. Nontreponemal tests lack sensitivity in late stage of infection and screening with these tests alone may yield false positive reactions in various acute and chronic conditions in the absence of syphilis (biological false positive reactions).

Infectious Serology-Rapid

HIV 1 &2 Ab-Chromatography				
HIV - I Results	Non Reactive		Non Reactive	Immuno Chromatography
HIV-II Results	Non Reactive		Non Reactive	Immunochromatography
Hepatitis B Surface Antigen (HBsAg) -Rapid				
Hepatitis B Surface Antigen(Rapid)	Non Reactive		Non Reactive	Immunochromatography
HCV Antibody (Rapid) - Serum				
Hepatitis C Virus Antibody (Rapid)	Non Reactive		Non Reactive	Immunochromatography

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ITDOSE INFOSYSTEMS PVT. LTD.

Name	: Mr. ADITYA PRATAP SINGH	Sample ID	: A4977284
Age/Gender	: 31 Years/Male	Reg. No	: 0032502240042
Referred by	:	Collected On	: 24-Feb-2025 10:30 AM
Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Plasma-NaF(F)	Reported On	: 24-Feb-2025 06:58 PM
Client Code	: BGLDLP010	Report Status	: Final Report

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CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Glucose Fasting (F)	**103.00**	mg/dL	70-100	GOD / POD Method

Interpretation of Plasma Glucose based on **ADA guidelines** 2018

Diagnosis	Fasting Plasma Glucose(mg/dL)	2hrs Plasma Glucose(mg/dL)	HbA1c(%)	RBS(mg/dL)
Prediabetes	100-125	140-199	5.7-6.4	NA
Diabetes	> = 126	> = 200	> = 6.5	>=200(with symptoms)

Reference: Diabetes care 2018:41(suppl.1):S13-S27

*** End Of Report ***

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Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Whole Blood EDTA	Reported On	: 24-Feb-2025 06:08 PM
Client Code	: BGLDLP010	Report Status	: Final Report

ITDOSE INFOSYSTEMS PVT. LTD.

CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Glycated Hemoglobin (HbA1c)	5.70	%	Non Diabetic:4.5-5.7 Pre diabetic: 5.8-6.4 Diabetic:>= 6.5	HPLC
Mean Plasma Glucose	116.89	mg/dL	90 - 120 mg/dl : Good Control 121 - 150 mg/dl : Fair Control 151 - 180 mg/dl : Unsatisfactory Control > 180 mg/dl : Poor Control	Calculated

Reference range for Known Diabetics-

Below 6.5%	Good Control
6.5% - 7%	Fair Control
7.0% - 8%	Unsatisfactory Control
>8%	Poor Control
:Goal of therapy:<7.0 :Action suggested:>8.0	Therapeutic goals for glycemic control

Interpretation:

- Glycated hemoglobins (GHb), also called glycohemoglobins, are substances formed when glucose binds to hemoglobin, and occur in amounts proportional to the concentration of serum glucose. Since red blood cells survive an average of 120 days, the measurement of GHb provides an index of a person's average blood glucose concentration (glycemia) during the preceding 2-3 months. Normally, only 4% to 6% of hemoglobin is bound to glucose, while elevated glycohemoglobin levels are seen in diabetes and other hyperglycemic states
- Mean Plasma Glucose(MPG):This Is Mathematical Calculations Where Glycated Hb Can Be Approximately Correlated With Daily Mean Plasma Glucose Level.
- Mean Plasma Glucose (mg/dL) = 28.7 x A1C - 46.7(As per American Association of Diabetics).

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Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Serum	Reported On	: 24-Feb-2025 06:56 PM
Client Code	: BGLDLP010	Report Status	: Final Report

CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
C Reactive Protein (CRP)				
C Reactive protein-(CRP)	0.40	mg/L	0-3.3	Particle Enhanced Immunoturbidimetric Assay

Interpretation:
C-reactive protein (CRP) is produced by the liver. The level of CRP rises when there is inflammation throughout the body. It is one of a group of proteins called acute phase reactants that go up in response to inflammation. The levels of acute phase reactants increase in response to certain inflammatory proteins called cytokines. These proteins are produced by white blood cells during inflammation.

A positive test means you have inflammation in the body. This may be due to a variety of conditions, including:

- Connective tissue disease
- Heart attack
- Infection
- Inflammatory bowel disease (IBD)
- Lupus
- Pneumonia
- Rheumatoid arthritis

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CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Amylase-Serum				
Amylase	52	U/L	25 – 115	CNP - G3

Interpretation :
Amylase is an enzyme that helps digest carbohydrates. It is made in the pancreas and the glands that make saliva. When the pancreas is diseased or inflamed, amylase releases into the blood.
This test is most often used to diagnose or monitor acute pancreatitis. It may also detect some digestive tract problems.
The test may also be done for the following conditions:

- Chronic pancreatitis
- Pancreatic pseudocys

Increased blood amylase level may occur due to:

- Acute pancreatitis
- Cancer of the pancreas, ovaries, or lungs
- Cholecystitis
- Gallbladder attack caused by disease
- Gastroenteritis (severe)
- Infection of the salivary glands (such as mumps) or a blockage

Decreased amylase level may occur due to:

- Cancer of the pancreas
- Damage to the pancreas with pancreatic scarring
- Kidney disease
- Toxemia of pregnancy

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ITDOSE INFOSYSTEMS PVT. LTD.

CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Lipid Profile				
Cholesterol Total	**261**	mg/dL	Desirable < 200~Borderline High 200 –239~ High ≥ 240	CECO HPO
Triglycerides-TGL	**156**	mg/dL	< 150-Normal 150-199 Boderline High 200-499 High >=500 Very High	Enzymatic Colorimetric
Cholesterol-HDL	**63**	mg/dL	40-60	Accelerator Selective Detergent
Cholesterol-LDL	**167**	mg/dL	Near optimal/above optimal 100-129 Borderline High 130-159 High 160-189 Very High ≥190	Calculated
Cholesterol- VLDL	**31.2**	mg/dL	< 30	Calculated
Non HDL Cholesterol	**198**	mg/dL	< 130	Calculated
LDL:HDL Ratio	2.65	%	0-3.5	Calculated
Total Cholesterol/HDL Cholesterol Ratio	4.14	Ratio	Low Risk: 3.3 - 4.4 Average Risk: 4.5 - 7.0 Moderate Risk: 7.1 - 11.0 High Risk: >=11	Calculated

The National Cholesterol Education program's third Adult Treatment Panel (ATPIII) has issued its recommendations on evaluating and treating lipid discorders for primary and secondary.

NCEP Recommendations	Cholesterol Total in (mg/dL)	Triglycerides in (mg/dL)	LDL Cholesterol in (mg/dL)	Non HDL Cholesterol in (mg/dL)
Desireable	Adult: < 200	< 150	Adult:<100	<130
Above Optimal	-----	-----	100-129	130 - 159
Borderline High	Adult: 200-239	150-199	Adult: 130-159	160 - 189
High	Adult:>or=240	200-499	Adult:160-189	190 - 219
Very High	-----	>or=500	Adult: >or=190	>=220

Note: LDL cholesterol cannot be calculated if triglyceride is >400 mg/dL (**Friedewald's formula**). Calculated values not provided for LDL and VLDL
Estimation of **LDL Cholesterol by direct method** is recommended when Triglyceride **>400 mg/dl.**

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ITDOSE INFOSYSTEMS PVT. LTD.

CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Kidney Profile				
Blood Urea Nitrogen (BUN)	12.0	mg/dL	6.0-20.0	Urease UV
Creatinine	1.11	mg/dL	0.70-1.30	Picrate Method
Sodium	139	mmol/L	136-145	ISE Direct
Potassium	5.0	mmol/L	3.5-5.5	ISE Direct
Chloride	104	mmol/L	98-108	ISE Direct
Glomerular Filtration Rate (GFR)	88.00	mL/min	Refer to Interpretation.	Calculated(MDRD Equation)
Uric Acid	4.70	mg/dL	2.6-6.0	Uricase
Calcium	9.77	mg/dL	8.1-10.4	Arsenazo III
Phosphorus(PO4)	3.50	mg/dL	2.5-4.9	Molybdate UV
Urea	26	mg/dL	15-39	Urease Colorimetric
BUN / Creatinine Ratio	10.81	Ratio	9:1-23:1	Calculated
Urea / Creatinine Ratio	23.14	Ratio	<52	Calculated

Reference range(eGFR):

Age(Years)	Average eGFR((mL/min/1.73 m2)	Age(Years)	Average eGFR((mL/min/1.73 m2)
20-29	116	50-59	93
30-39	107	60-69	85
40-49	99	>=70	75

Chronic kidney disease:< 60 mL/min/1.73 m2 **Kidney failure:** <15 mL/min/1.73 m2

Interpretation:

The kidneys, located in the retroperitoneal space in the abdomen, are vital for patient health. They process several hundred liters of fluid a day and remove around two liters of waste products from the bloodstream. The volume of fluid that passes though the kidneys each minute is closely linked to cardiac output. The kidneys maintain the body's balance of water and concentration of minerals such as sodium, potassium, and phosphorus in blood and remove waste by-products from the blood after digestion, muscle activity and exposure to chemicals or medications. They also produce renin which helps regulate blood pressure, produce erythropoietin which stimulates red blood cell production, and produce an active form of vitamin D, needed for bone health.

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CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Liver Function Test (LFT)				
Bilirubin(Total)	0.75	mg/dL	0.0-1.00	DPD
Bilirubin (Direct)	0.06	mg/dL	0.0 - 0.3	Modified Malloy-Evelyn
Bilirubin (Indirect)	0.69	mg/dL	0.2-0.8	Calculated
Aspartate Aminotransferase (AST/SGOT)	28	U/L	15-37	IFCC without (P-5-P)
Alanine Aminotransferase (ALT/SGPT)	55	U/L	16-63	IFCC without (P-5-P)
Alkaline Phosphatase(ALP)	98	U/L	40 - 129	IFCC
Gamma Glutamyl Transferase (GGT)	25.0	U/L	15-85	IFCC
Protein - Total	8.20	g/dL	6.4-8.2	Biuret
Albumin	5.00	g/dL	3.4-5.00	Bromo cresol purple (BCP)
Globulin	3.20	g/dL	2.0-3.5	Calculated
A:G Ratio	1.56	%	1.0-2.1	Calculated
AST/ALT Ratio	0.51	Ratio	<1	Calculated

- The liver serves several essential functions that support a person's overall health and well-being. It removes toxins from the blood, metabolizes fats and proteins, and regulates blood clotting
- **Alanine Aminotransferase(ALT**) is an enzyme found in liver and kidneys cells. ALT helps create energy for liver cells. Damaged liver cells release ALT into the bloodstream, which can elevate ALT levels in the blood.
- **Aspartate Aminotransferase (AST)** is an enzyme in the liver and muscles that helps metabolizes amino acids. Similarly to ALT, elevated AST levels may be a sign of liver damage or liver disease.
- **Alkaline phosphate (ALP)** is an enzyme present in the blood. ALP contributes to numerous vital bodily functions, such as supplying nutrients to the liver, promoting bone growth, and metabolizing fat in the intestines.
- **Gamma-glutamyl Transpeptidase (GGTP)** is an enzyme that occurs primarily in the liver, but it is also present in the kidneys, pancreas, gallbladder, and spleen. Higher than normal concentrations of GGTP in the blood may indicate alcohol-related liver damage. Elevated GGTP levels can also increase the risk of developing certain types of cancer.
- **Bilirubin** is a waste product that forms when the liver breaks down red blood cells. Bilirubin exits the body as bile in stool. High levels of bilirubin can cause jaundice - a condition in which the skin and whites of the eyes turn yellow- and may indicate liver damage.
- **Albumin** is a protein that the liver produces. The liver releases albumin into the bloodstream, where it helps fight infections and transport vitamins, hormones, and enzymes throughout the body. Liver damage can cause abnormally low albumin levels.

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Sample Type	: Serum	Reported On	: 24-Feb-2025 06:56 PM
Client Code	: BGLDLP010	Report Status	: Final Report

ITDOSE INFOSYSTEMS PVT. LTD.

CLINICAL BIOCHEMISTRY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Iron Profile-I				
Iron(Fe)	117	µg/dL	65-175	Ferrozine
Unsaturated iron binding capacity (UIBC)	222	µg/dL	155 – 300	Calculated
Total Iron Binding Capacity (TIBC)	339	µg/dL	250-450	Ferene
Transferrin	237.06	mg/dL	215-365	Calculated
Iron Saturation((% Transferrin Saturation)	34.51	%	20-50	Calculated

Interpretation:

- Serum transferrin (and TIBC) high, serum iron low, saturation low. Usual causes of depleted iron stores include blood loss, inadequate dietary iron. RBCs in moderately severe iron deficiency are hypochromic and microcytic. Stainable marrow iron is absent. Serum ferritin decrease is the earliest indicator of iron deficiency if inflammation is absent.
- **Anemia of chronic disease:** Serum transferrin (and TIBC) low to normal, serum iron low, saturation low or normal. Transferrin decreases with many inflammatory diseases. With chronic disease there is a block in movement to and utilization of iron by marrow. This leads to low serum iron and decreased erythropoiesis. Examples include acute and chronic infections, malignancy and renal failure.
- **Sideroblastic Anemia:** Serum transferrin (and TIBC) normal to low, serum iron normal to high, saturation high.
- **Hemolytic Anemia:** Serum transferrin (and TIBC) normal to low, serum iron high, saturation high.
- **Hemochromatosis:** Serum transferrin (and TIBC) slightly low, serum iron high, saturation very high.
- **Protein depletion:** Serum transferrin (and TIBC) may be low, serum iron normal or low (if patient also is iron deficient). This may occur as a result of malnutrition, liver disease, renal disease.
- **Liver disease:** Serum transferrin variable; with acute viral hepatitis, high along with serum iron and ferritin. With chronic liver disease (eg, cirrhosis), transferrin may be low. Patients who have cirrhosis and portacaval shunting have saturated TIBC/transferrin as well as high ferritin.

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Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Urine	Reported On	: 24-Feb-2025 05:12 PM
Client Code	: BGLDLP010	Report Status	: Final Report

CLINICAL PATHOLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Complete Urine Analysis (CUE)				
Physical Examination				
Colour	Pale Yellow		Straw to light amber	Visual Examination
Appearance	Clear		Clear	Visual Examination
Chemical Examination				
Glucose	Negative		Negative	Strip Reflectance (GODPOD) Benedicts
Protein	Absent		Negative	Error of indicators principle
Ketone Bodies	Negative		Negative	Strip Reflectance -Sodium nitroprusside
Specific Gravity	1.025		1.005 - 1.030	pKa change
Blood	Negative		Negative	Strip Reflectance (Diazonum)
Reaction (pH)	6.0		5.0 - 8.5	Double indicator Principle
Bilirubin (Bile)	Negative		Negative	Azo-coupling reaction
Urobilinogen	Normal		Normal	Reagent strip Reflectance - Modified Ehrlichs Reaction
Nitrites	Negative		Negative	Strip Reflectance (Diazonum)
Leucocyte	Positive	-	Negative	Strip Reflectance (Phenylpyrrole and Diazonium)
Microscopic Examination				
PUS(WBC) Cells	5-7	/HPF	1-2	Microscopy
R.B.C	Nil	/HPF	0-2	Microscopy
Epithelial Cells	2-4	/HPF	1-2	Microscopy
Casts	Absent	/HPF	Absent	Microscopy
Crystals	Nil	/HPF	Absent	Microscopy
Bacteria	Nil	/HPF	Nil	Microscopy

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Sample Type	: Urine	Reported On	: 24-Feb-2025 05:12 PM
Client Code	: BGLDLP010	Report Status	: Final Report

CLINICAL PATHOLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Others	-	/HPF	Nil	Microscopy

Glucose:
Moderate amounts of ketone bodies (40mg/dL or greater) may decrease color development in urine containing small amounts of glucose (75-125 mg/dl). However, such concentration of ketone simultaneously with such glucose concentration is metabolically improbable in screening. The reactivity of the glucose test decreases as the SG and/or ascorbic acid of the urine increases. Reactivity may also vary with temperature.

Bilirubin:
Reactions may occur with urine containing large doses of chlorpromazine or rafampen that might be mistaken for positive bilirubin. 3 Indican (indoxyl sulfate) and metabolites of Cortez may cause false positive or atypical color; ascorbic acid (25mg/dL or greater) may cause false negative results.

Ketone:
Color reaction that could be interpreted as “positive” may be obtained with urine specimens containing MESNA or large amounts of phenylketones or L-dopa metabolites.

Specific Gravity:
The chemical nature of the specific gravity test may cause slightly different results from those obtained with the specific gravity methods when elevated amounts of certain urine constituents are present. Highly buffered alkaline urine may cause low readings relative to other methods. Elevated specific gravity readings may be obtained in the presence of moderate quantities (100-750 mg/dl) of protein

Blood:
The sensitivity of the blood test is reduced in urine with high specific gravity and/or high ascorbic acid content. Microbial peroxidase, associated with urinary tract infection may cause false positive reactions.

pH:
If proper procedure is not followed and excess urine remains on the strip, a phenomenon known as “running over” may occur, in which the acid buffer from the protein reagent area run onto the pH area, causing a false lowering in the pH result.

Protein:
False positive results may be obtained with highly alkaline urine. Contamination of the urine specimen with quarternary ammonium compounds may also produce false positive results.

Urobilinogen:
The test area will react with interfering substances known to react with Ehrlich’s reagent, such as porphobilinogen and p-aminosalicyclic acid. 3 This test is not a reliable method for the detection of porphobilinogen. Drugs containing azo-dyes (Cortez) may give a masking golden color. The absence of urobilinogen cannot be determined with this test

Nitrite:
The pink color is not quantitative in relation to the number of bacteria present. Any degree of pink coloration should be interpreted as a positive nitrite test suggestive of 105 or more organisms/ml. There are occasional urinary tract infections from organisms, which do not contain reductase to convert nitrate to nitrite.

Leukocytes:
Highly colored urine and the presence of the drugs cephalexin (Cortez) and gentamicin have been found to interfere with this test. High urinary protein of 500 mg/dl or above diminishes the intensity of the reaction color. Elevated glucose concentration or high specific gravity may cause decreased results.

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Referred by	:	Collected On	: 24-Feb-2025 10:30 AM
Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Whole Blood EDTA	Reported On	: 24-Feb-2025 05:59 PM
Client Code	: BGLDLP010	Report Status	: Final Report

HAEMATOLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
COMPLETE BLOOD COUNT (CBC)				
Haemoglobin (Hb)	16.2	g/dL	13-17	Cyanide free Colorimetry
RBC Count	**6.61**	10^12/L	4.5-5.5	Electric Impedance
Packed Cell Volume	**53.60**	%	40-50	Calculated
MCV	**81.0**	fl	83-101	Calculated
MCH	**24.5**	pg	27-32	Calculated
MCHC	**30.2**	g/dL	32.5-34.5	Calculated
RDW-CV	13.3	%	11.6-14.0	Calculated
Platelet Count (PLT)	177	10^9/L	150-410	Electric Impedance
Total WBC Count	6.68	10^9/L	4.0-10.0	Electric Impedance
Differential Cell Count				
Neutrophils	42.1	%	40-80	Flow cytometry / Microscopy
Absolute Neutrophils Count	2.81	10^9/L	2.0-7.0	Calculated
Lymphocytes	**48.4**	%	20-40	Flow cytometry / Microscopy
Absolute Lymphocyte Count	**3.23**	10^9/L	1.0-3.0	Calculated
Monocytes	4.8	%	2-10	Flow cytometry / Microscopy
Absolute Monocyte Count	0.32	10^9/L	0.2-1.0	Calculated
Eosinophil	4.5	%	1-6	Flow cytometry / Microscopy
Absolute Eosinophils Count	0.3	10^9/L	0.02-0.5	Calculated
Basophils	0.2	%	1-2	Flow cytometry / Microscopy
Absolute Basophil Count	**0.0**	10^9/L	0.02-0.10	Calculated
Other Cells	0.0	%	-	Microscopy

*** End Of Report ***

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ITDOSE INFOSYSTEMS PVT. LTD.

Name	: Mr. ADITYA PRATAP SINGH	Sample ID	: A4977281
Age/Gender	: 31 Years/Male	Reg. No	: 0032502240042
Referred by	:	Collected On	: 24-Feb-2025 10:30 AM
Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Whole Blood EDTA	Reported On	: 24-Feb-2025 05:59 PM
Client Code	: BGLDLP010	Report Status	: Final Report

HAEMATOLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Erythrocyte Sedimentation Rate (ESR)				
Erythrocyte Sedimentation Rate (ESR)	5	mm/hr	10 or less	Westergren method

Comments :

ESR is an acute phase reactant which indicates presence and intensity of an inflammatory process.It is never diagnostic of a specific disease. It is used to monitor the course or response to treatment of certain diseases. Extremely high levels are found in cases of malignancy, hematologic diseases, collagen disorders and renal diseases.

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Referred by	:	Collected On	: 24-Feb-2025 10:30 AM
Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Serum	Reported On	: 24-Feb-2025 05:49 PM
Client Code	: BGLDLP010	Report Status	: Final Report

ITDOSE INFOSYSTEMS PVT. LTD.

IMMUNOLOGY & SEROLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
25 - Hydroxy Vitamin D3	**21.1**	ng/mL	<20.0-Deficiency 20.0-<30.0-Insufficiency 30.0-100.0-Sufficiency >100.0-Potential Intoxication	CLIA

Interpretation:

- Vitamin D helps your body absorb calcium and maintain strong bones throughout your entire life. Your body produces vitamin D when the sun's UV rays contact your skin. Other good sources of the vitamin include fish, eggs, and fortified dairy products. It's also available as a dietary supplement.
- Vitamin D must go through several processes in your body before your body can use it. The first transformation occurs in the liver. Here, your body converts vitamin D to a chemical known as 25-hydroxyvitamin D, also called calcidiol.
- The 25-hydroxy vitamin D test is the best way to monitor vitamin D levels. The amount of 25-hydroxyvitamin D in your blood is a good indication of how much vitamin D your body has. The test can determine if your vitamin D levels are too high or too low.
- .The test is also known as the 25-OH vitamin D test and the calcidiol 25-hydroxycholecalcifoerol test. It can be an important indicator of osteoporosis (bone weakness) and rickets (bone malformation).

Those who are at high risk of having low levels of vitamin D include:

- people who don't get much exposure to the sun
- older adults
- people with obesity.
- dietary deficiency

Increased Levels:

- Vitamin D Intoxication

*** End Of Report ***

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Age/Gender	: 31 Years/Male	Reg. No	: 0032502240042
Referred by	:	Collected On	: 24-Feb-2025 10:30 AM
Referring Customer	:	Registered On	: 24-Feb-2025 03:50 PM
Sample Type	: Serum	Reported On	: 24-Feb-2025 08:20 PM
Client Code	: BGLDLP010	Report Status	: Final Report

ITDOSE INFOSYSTEMS PVT. LTD.

IMMUNOLOGY & SEROLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Vitamin- B12 (cyanocobalamin)	349	pg/mL	211-911	CLIA

This test is most often done when other blood tests suggest a condition called megaloblastic anemia. Pernicious anemia is a form of megaloblastic anemia caused by poor vitamin B12 absorption. This can occur when the stomach makes less of the substance the body needs to properly absorb vitamin B12.
Causes of vitamin B12 deficiency include:Diseases that cause malabsorption

- Lack of intrinsic factor, a protein that helps the intestine absorb vitamin B12
- Above normal heat production (for example, with hyperthyroidism)

An increased vitamin B12 level is uncommon in:

- Liver disease (such as cirrhosis or hepatitis)
- Myeloproliferative disorders (for example, polycythemia vera and chronic myelogenous leukemia)

Total IgE	7.54	IU/mL	<100	Chemiluminescent
Ferritin	128.00	ng/mL	20-250	Chemiluminescent

Interpretation:
The ferritin blood test measures the level of ferritin in the blood.
Ferritin is a protein inside your cells that stores iron. It allows your body to use the iron when it needs it. A ferritin test indirectly measures the amount of iron in your blood.
A higher-than-normal ferritin level may be due to:

- Liver disease due to alcohol abuse
- Any autoimmune disorder, such as rheumatoid arthritis
- Frequent transfusion of red blood cells

A lower-than-normal level of ferritin occurs if you have anemia caused by low iron levels in the body. This type of anemia may be due to:

- A diet too low in iron
- Heavy bleeding from an injury
- Heavy menstrual bleeding

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IMMUNOLOGY & SEROLOGY

MEDITEST ADVANCED FULL BODY CHECKUP-PLATINUM

Test Name	Results	Units	Bio. Ref. Intervel	Method
Thyroid Profile-I				
T3 (Triiodothyronine)	124.00	ng/dL	70-204	CLIA
T4 (Thyroxine)	7.81	µg/dL	3.2-12.6	CLIA
TSH (Thyroid Stimulating Hormone)	4.300	µIU/mL	0.35-5.5	CLIA

Pregnancy & Cord Blood

T3 (Triiodothyronine):	T4 (Thyroxine)	TSH (Thyroid Stimulating Hormone)
First Trimester : 0.81-1.90 ng/mL	15 to 40 weeks:9.1-14.0 µg/dL	First Trimester : 0.24-2.99 µIU/mL
Second&Third Trimester :1.00-2.60 ng/mL		Second Trimester: 0.46-2.95 µIU/mL
		Third Trimester : 0.43-2.78 µIU/mL
Cord Blood: 0.30-0.70 ng/mL	Cord Blood: 7.4-13.0 µg/dL	Cord Blood: : 2.3-13.2 µIU/mL

Interpretation:

- Thyroid gland is a butterfly-shaped endocrine gland that is normally located in the lower front of the neck. The thyroid's job is to make thyroid hormones, which are secreted into the blood and then carried to every tissue in the body. Thyroid hormones help the body use energy, stay warm and keep the brain, heart, muscles, and other organs working as they should.
- Thyroid produces two major hormones: triiodothyronine (T3) and thyroxine (T4). If thyroid gland doesn't produce enough of these hormones, you may experience symptoms such as weight gain, lack of energy, and depression. This condition is called hypothyroidism.
- Thyroid gland produces too many hormones, you may experience weight loss, high levels of anxiety, tremors, and a sense of being on a high. This is called hyperthyroidism.
- TSH interacts with specific cell receptors on the thyroid cell surface and exerts two main actions. The first action is to stimulate cell reproduction and hypertrophy. Secondly, TSH stimulates the thyroid gland to synthesize and secrete T3 and T4.
- The ability to quantitate circulating levels of TSH is important in evaluating thyroid function. It is especially useful in the differential diagnosis of primary (thyroid) from secondary (pituitary) and tertiary (hypothalamus) hypothyroidism. In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels are low.

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